

SynthMicro — Whole Blood Smear XAI Report

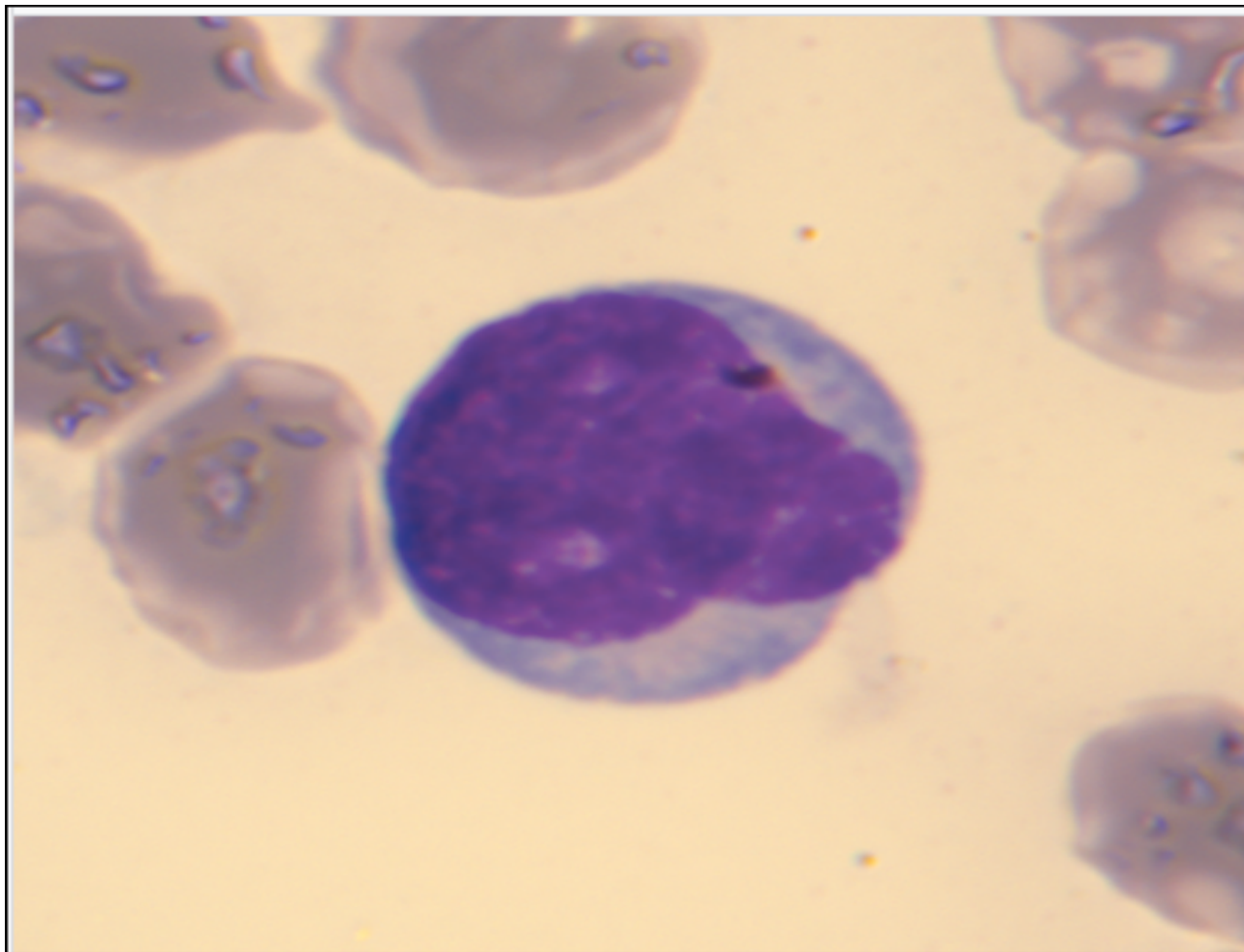
Analysis ID: f6bcbdd41b3364db6aa2e108d8b6fda82
Generated: 2026-09-07 20:41:23

Pipeline: Cellpose segmentation → 384×384 cell extraction → ResNet50 classification → Grad-CAM → SHAP → explanatory report.

Measurement	Result
Cellpose objects	1
Nucleated-cell candidates	1
Myeloblast-like candidates	1
Myeloblast-like proportion	100.00%

Screening interpretation: the proportion above is a research-oriented model indicator based on candidate cells selected by the notebook's purple-score rule. It is not a cancer probability and does not establish AML or another diagnosis.

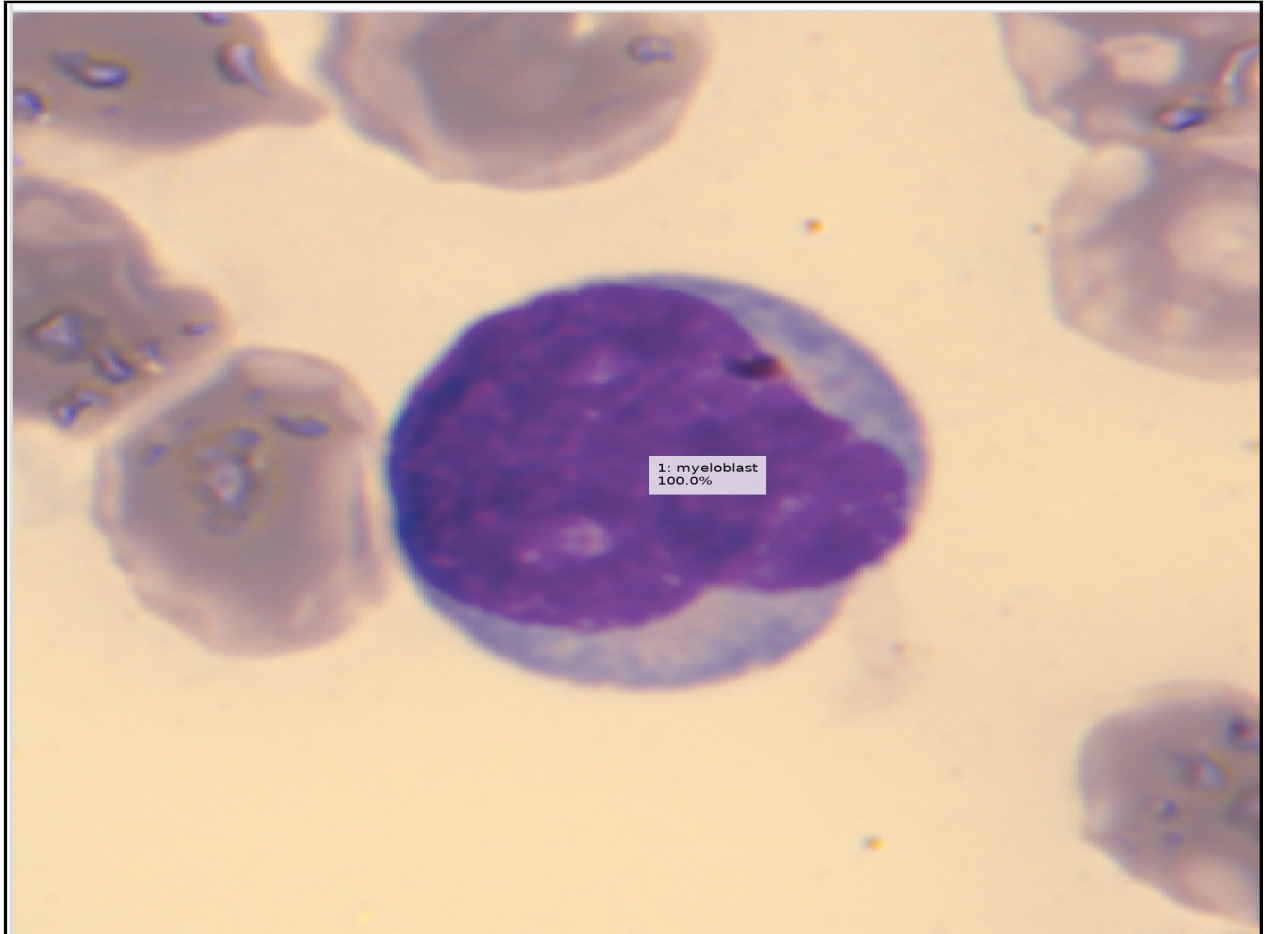
Whole Smear



Original uploaded blood-smear image analyzed by Cellpose.

Segmentation and Candidate Overlay

Whole Smear — Candidate Cell Classification



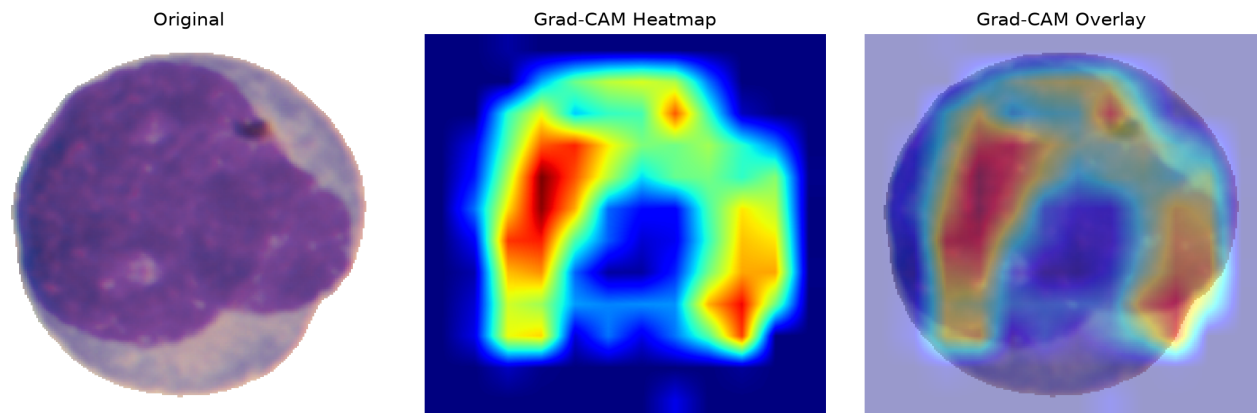
Cell-Level Results

ID	Area	Purple	Class	Confidence
1	26655	47.6	myeloblast	100.0%

Grad-CAM Explanations

Grad-CAM shows regions contributing to the selected model class. It is an explanation of model behavior, not a direct measurement of a biological structure.

Cell 1 — myeloblast (100.0%)



The ResNet50 model classified this segmented candidate as a myeloblast with 100.0% confidence. This is a model-level cell classification; a myeloblast-like prediction alone does not establish AML or another cancer diagnosis.

SHAP Explanations

SHAP attribution magnitude summarizes the magnitude of pixel-level contribution toward the selected class. It is a model explanation, not a clinical measurement.

SHAP output was unavailable for this analysis.

Interpretation and Limitations

- The classifier has five closed-set classes: basophil, erythroblast, monocyte, myeloblast and segmented neutrophil.
- RBCs and unknown cell types are not explicit classes and can therefore be forced into one of the five outputs.
- Cellpose segmentation quality affects every downstream result.
- Stain, microscope, illumination and acquisition differences can cause domain shift.
- Confidence is not a calibrated probability of disease.
- A myeloblast-like prediction is not equivalent to an AML diagnosis.
- Clinical assessment requires appropriate hematopathology and laboratory testing.